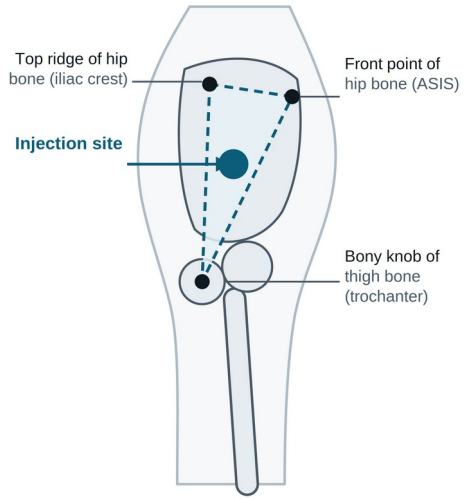
Testosterone undecanoate 1000 mg/4 mL · treatment-room card

Ventrogluteal IM injection · observe · act. Print and laminate. Version 2.1, 16 October 2026. Full SOP: https://gamsa.au/medicines/testosterone-undecanoate

Where the long-acting injection goes

The ventrogluteal site — the side of the hip



1. Palm flat on the bony knob, fingers pointing up
2. Index finger on the front point of the hip bone
3. Spread your middle finger back along the top ridge
- Needle into the V between them, straight in at 90°

1 Screen

Identity, consent, allergies, prior reaction to oily injection [1].

Defer or refer if: chest infection or breathless; previous POME; bleeding disorder or anticoagulant [1][11].

Recommended: pregnancy test before each injection if the patient could become pregnant [1][12].

Product, 1000 mg/4 mL, batch, expiry.

2 Equipment

Draw up **18G**. Inject **21G × 38 mm**; **50 mm if BMI > 30** [5][10].

Oxygen, adrenaline, monitoring, phone for 000 in the room [8].

3 Landmark and inject

Side-lying, upper hip relaxed. Heel of hand on greater trochanter, index finger to ASIS, middle finger along iliac crest: inject in the **V** [4][5].

90°, deep into muscle. **Aspirate**; blood → withdraw, new needle, new site [5][10].

Full 4 mL **very slowly, about 2 minutes** [1][10].

Gentle pressure, dressing. **No massage** [5].

4 Observe

During and immediately after [1][3], and for **at least 30 minutes** under this procedure: POME typically presents within 30 minutes [12]; the PI gives no fixed time.

Record: product, batch, expiry, side, needle, time taken, clinician, outcome, next due (10–14 weeks; first interval ≥ 6 weeks) [1][3].

Book the next appointment before the patient leaves.

POME: cough, urge to cough, breathless, chest tight, sweating, dizzy, tingling, faint — within minutes

Stop. Sit up, legs out. **Oxygen.** Monitor pulse, BP, SpO_2, RR [7].

Call **000** immediately for severe breathing difficulty, collapse or signs of anaphylaxis [7][8].

Mild POME symptoms usually settle within 30 minutes to a few hours with supportive care [6][12].

ANAPHYLAXIS: rash, swelling, wheeze, stridor, hypotension, collapse

Lie flat, legs raised; sit with legs out only if breathing is the problem. Do not stand [8].

Adrenaline 1:1000 IM, outer mid-thigh: 0.01 mg/kg, max 0.5 mg (0.5 mL). Call **000**. Repeat after 5 min if no better [8].

Oxygen. Hospital observation ≥ 4 h after last dose [8].

Afterwards

Notify prescriber. Adverse event form. Report to TGA: **aems.tga.gov.au** [9].

References by number are in the SOP and training guide. Prepared by Theodore South, BPharm (Hons); a template for adaptation to local policy, not endorsed by any health service. Product Information prevails [1][11].